Zylk Health

Thyrocare

Tests you can trust

Name : Kanagavalli K(21Y/F)

Date : 15 Sep, 2026

Test Asked : Jaanch - Womens Hairfall Screening Advanced

Report Status : Complete Report







Processed At :
UR House, No. 1056C, Avinashi Road, Coimbatore - 641018

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Patient Name	: **KANAGAVALLI K (21Y/F)**	Tests Done	: **JAANCH - WOMENS HAIRFALL SCREENING ADVANCED**
Referred By	: **SELF**		
Address	: **INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007**		

Report Availability Summary

Note: Please refer to the table below for status of your tests.

20 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
JAANCH - WOMENS HAIRFALL SCREENING ADVANCED	Ready
CALCIUM	Ready
CORTISOL	Ready
C-REACTIVE PROTEIN (CRP)	Ready
FERRITIN	Ready
FOLLICLE STIMULATING HORMONE (FSH)	Ready
VDRL (RPR) FOR SYPHILIS	Ready
LUTEINISING HORMONE (LH)	Ready
MAGNESIUM	Ready
PHOSPHOROUS	Ready
PROLACTIN (PRL)	Ready
SERUM ZINC	Ready
SEX HORMONE BINDING GLOBULIN (SHBG)	Ready
TOTAL TRIIODOTHYRONINE (T3)	Ready
TOTAL THYROXINE (T4)	Ready
TESTOSTERONE	Ready
TSH - ULTRASENSITIVE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
IRON DEFICIENCY PROFILE	Ready
VITAMIN D TOTAL AND B12 COMBO	Ready
ANTI NUCLEAR ANTIBODIES (ANA)	Ready

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Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
ARTHRITIS			
PHOSPHOROUS	**10.27**	mg/dL	2.4 - 5.1
COMPLETE HEMOGRAM			
EOSINOPHILS	**6.7**	%	1-6
EOSINOPHILS - ABSOLUTE COUNT	**0.56**	X 10^3 / µL	0.02 - 0.5
HEMOGLOBIN	**9.7**	g/dL	12.0-15.0
MEAN CORP.HEMO.CONC(MCHC)	**25.1**	g/dL	31.5-34.5
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	**19.9**	pq	27.0-32.0
MEAN CORPUSCULAR VOLUME(MCV)	**79.5**	fL	83.0-101.0
PLATELET DISTRIBUTION WIDTH(PDW)	**9.4**	fL	9.6-15.2
PLATELET TO LARGE CELL RATIO(PLCR)	**17.9**	%	19.7-42.4
RED CELL DISTRIBUTION WIDTH (RDW-CV)	**21.4**	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	**60.5**	fL	39.0-46.0
TOTAL RBC	**4.87**	X 10^6/µL	3.8-4.8
INFERTILITY			
PROLACTIN (PRL)	**54.8**	ng/mL	4.79-23.3
IRON DEFICIENCY			
% TRANSFERRIN SATURATION	**5.88**	%	13 - 45
IRON	**28.2**	µg/dL	37 - 165
UNSAT.IRON-BINDING CAPACITY(UIBC)	**451.8**	µg/dL	162 - 368
METABOLIC			
MAGNESIUM	**2.61**	mg/dL	1.9 - 2.5
THYROID			
TSH - ULTRASENSITIVE	**21.747**	µIU/mL	0.35-4.94
VITAMINS			
25-OH VITAMIN D (TOTAL)	**11.4**	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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Introducing

Health Trends

All your health reports, in one place — watch how things change over time.

OTP: 3798

CLICK or SCAN to access detailed trends

Test Name	Test Result & Bio. Ref. Interval
Thyroid Function	**15 Sep '26**
TSH - ULTRASENSITIVE	**21.747 µIU/mL** 0.35 - 4.94
TOTAL TRIIODOTHYRONINE (T3)	76 ng/dL 58 - 159
TOTAL THYROXINE (T4)	6.1 µg/dL 4.87 - 11.72
Blood Health & Iron Status	**15 Sep '26**
HEMOGLOBIN	**9.7 g/dL** 12 - 15
IRON	**28.2 µg/dL** 37 - 165
% TRANSFERRIN SATURATION	**5.88 %** 13 - 45
PLATELET COUNT	376 X 10^3 / µL 150 - 410

+4 more tests (Use above QR for detailed trend)

Vitamins, Electrolytes & Minerals	15 Sep '26
VITAMIN B-12	343 pg/mL 197 - 771
25-OH VITAMIN D (TOTAL)	**11.4 ng/mL** 30 - 100
CALCIUM	9.28 mg/dL 8.6 - 10.3
MAGNESIUM	**2.61 mg/dL** 1.9 - 2.5

Trends Unlock After Your Next Test

That's all for now!

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Test Name	Test Result & Bio. Ref. Interval	Trends Unlock After Your Next Test
Hormonal Health	**15 Sep '26**	
TESTOSTERONE	37 ng/dL 6 - 82	
PROLACTIN (PRL)	**54.8 ng/mL** 4.79 - 23.3	
FOLLICLE STIMULATING HORMONE (FSH)	4.33 mIU/mL 1.7 - 134.8	
LUTEINISING HORMONE (LH)	10.7 mIU/mL 1 - 95.6	

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Patient Name	: KANAGAVALLI K (21Y/F)	**Sample Collected on (SCT)**	: 15 Sep 2026 11:12
Referred By	: SELF	**Sample Received on (SRT)**	: 15 Sep 2026 11:53
Address	: INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007	**Report Released on (RRT)**	: 16 Sep 2026 09:05
		Sample Type | Barcode	: SERUM | FN883087

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**C.M.I.A**	**11.4**	**ng/mL**

Bio. Ref. Interval. :
DEFICIENCY : <10 ng/ml
INSUFFICIENCY : 10 - 30 ng/ml
SUFFICIENCY : 30 - 100 ng/ml
TOXICITY : >100 ng/ml

Method : Fully Automated Chemi Luminescent Microparticle Immunoassay

VITAMIN B-12	E.C.L.I.A	343	pg/mL

Bio. Ref. Interval. :
Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Method : Fully Automated Electrochemiluminescence Compititive Immunoassay

Please correlate with clinical conditions.

Tests Done : JAANCH - WOMENS HAIRFALL SCREENING ADVANCED

Report Remarks : LabCode:1509040738/FE254

Dr.Vinitha Jose
MD(Path)

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TEST NAME	TECHNOLOGY	VALUE
VDRL (RPR) FOR SYPHILIS	FLOCCULATION	NON REACTIVE

Clinical Significance :-

Syphilis is a sexually transmitted disease caused by a spirochete Treponema pallidum. It can cause long-term complications by invading the nervous and cardiovascular system, if not adequately treated. It may also be transmitted from mother to baby during pregnancy or at birth, resulting in congenital syphilis.

Interpretation:

RPR test is an effective screening test for syphilis. The test antigen is a modified form of VDRL antigen containing microparticulate carbon, which aids the macroscopic reading of results. RPR test may give false positive results in patients suffering from HIV, tuberculosis, leprosy, infectious mononucleosis and any autoimmune disease. Weak reactive and Reactive results must be confirmed using Treponema pallidum Hemagglutination Assay(TPHA) and fluorescent treponemal antibody absorption (FTA-ABS).

References:

Manual test for Syphilis Phs Publications No 411, (1969)

Please correlate with clinical conditions.
Method:- N/A

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.M.I.A	76	ng/dL	58-159
TOTAL THYROXINE (T4)	C.M.I.A	6.1	µg/dL	4.87-11.72
TSH - ULTRASENSITIVE	**C.M.I.A**	**21.747**	**µIU/mL**	**0.35-4.94**

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4,USTSH - Fully Automated Chemi Luminescent Microparticle Immunoassay

Pregnancy reference ranges for TSH/USTSH :
Trimester || T3 (ng/dl) || T4 (µg/dl) || TSH/USTSH (µIU/ml)
1st || 83.9-196.6 || 4.4-11.5 || 0.1-2.5
2nd || 86.1-217.4 || 4.9-12.2 || 0.2-3.0
3rd || 79.9-186 || 5.1-13.2 || 0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	**PHOTOMETRY**	**28.2**	**µg/dL**
Bio. Ref. Interval. : 37 - 165			
Method : Photometric test using Ferene			
TOTAL IRON BINDING CAPACITY (TIBC)	PHOTOMETRY	480	µg/dL
Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl			
Method : Spectrophotometric Assay			
% TRANSFERRIN SATURATION	**CALCULATED**	**5.88**	**%**
Bio. Ref. Interval. : 13 - 45			
Method : Derived from IRON and TIBC values			
UNSAT.IRON-BINDING CAPACITY(UIBC)	**PHOTOMETRY**	**451.8**	**µg/dL**
Bio. Ref. Interval. : 162 - 368			
Method : SPECTROPHOTOMETRIC ASSAY			
FERRITIN	C.M.I.A	10.4	ng/mL
Bio. Ref. Interval. : Men: 21.81 - 274.66 ng/ml Women: 4.63 - 204.00 ng/ml			
Method : Fully Automated Chemi Luminescent Microparticle Immunoassay			

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
TESTOSTERONE	E.C.L.I.A	37	ng/dL

Bio. Ref. Interval. :-

6 - 82

Clinical Significance: Clinical evaluation of serum testosterone, along with serum LH, assists in evaluation of Hypogonadal males. Major causes of lowered testosterone in males include Hypogonadotropic hypogonadism, testicular failure Hyperprolactinema, Hypopituitarism some types of liver and kidney diseases and critical illness.

Specifications: Precision: Intra assay (%CV): 11.50 %, Inter assay (%CV): 5.70%; Sensitivity: 7 ng/dL.
Kit Validation Reference: Wilson JD Foster DW (Eds) Williams Textbook of Endocrinology 8th Edition WB Saunders Piladelphia Pennsylvania.

Note : The Biological Reference Range mentioned is specific to the age group and gender. Kindly correlate clinically.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SEX HORMONE BINDING GLOBULIN (SHBG)	C.L.I.A	36.4	nmol/L

Bio. Ref. Interval. :-

Males 10 - 57 nmol/L

Females
Non-Pregnant : 18 - 144

Clinical Significance:

Sex hormone binding globulin (SHBG) has a high affinity for testosterone and Estradiol, and is a major factor regulating their distribution between the protein-bound and free states. The ratio of testosterone to SHBG is also known as the free androgen index (FAI) or the free testosterone index (FTI). This ratio correlates well with both measured and calculated values of free testosterone and helps to discriminate subjects with excessive androgen activity from normal individuals. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications:

Precision: Intra assay (%CV): 5.30 %, Inter assay (%CV):6.60%
Sensitivity: < 0.02 nmol/l and Specificity: no detectable cross- reactivity

Kit validation reference:

Bond A, Davis C. Sex Hormone binding globulin in clinical perspective, ActaObset Gynecol Scand 1987;66:255-62

Please correlate with clinical conditions.
Method:- SOLID-PHASE TWO-SITE CHEMILUMINESCENT IMMUNOMETRIC ASSAY

Tests Done : JAANCH - WOMENS HAIRFALL SCREENING ADVANCED

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Saakshi

Dr Saakshi Mittal
MD(Path)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SERUM ZINC	PHOTOMETRY	124.4	µg/dL

Bio. Ref. Interval. :-

46 - 150

Clinical Significance:
Zinc is one of the essential trace elements in the body. Its metalloenzymes play a key role in protein and nucleic acid synthesis, gene expression, wound healing, as an antioxidant, etc. Deficiency can cause- Poor wound healing, gastroenteritis, impaired spermatogenesis, Alzheimer's disease, etc. Toxicity may be manifested as pancreatitis, gastric ulcer, anemia, pulmonary fibrosis.

Please correlate with clinical conditions.
Method:- BrPAPS

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TEST NAME	TECHNOLOGY	VALUE	UNITS
FOLLICLE STIMULATING HORMONE (FSH)	E.C.L.I.A	4.33	mIU/mL

Bio. Ref. Interval. :
Female :
Follicular Phase : 3.5 to 12.5 I Ovulate Phase : 4.7 to 21.5
Luteal Phase : 1.7 to 7.70 I Post Menopausal : 25.8 to 134.8
Male : 1.5 to 12.4

Method : Fully Automated Chemiluminescent Immunoassay

LUTEINISING HORMONE (LH)	E.C.L.I.A	10.7	mIU/mL

Bio. Ref. Interval. :
Female :
Follicular Phase : 2.4 to 12.6 I Ovulate Phase : 14 to 95.6
Luteal Phase : 1.0 to 11.4 I Post Menopausal : 7.7 to 58.5
Male : 1.7 to 8.6

Method : Sandwhich Electrochemiluminescence Immunoassay

PROLACTIN (PRL)	**E.C.L.I.A**	**54.8**	**ng/mL**

Bio. Ref. Interval. :
Men : 4.04-15.2 ng/ml
Women (Non Pregnant) : 4.79-23.3 ng/ml
First Trimester 9.95 - 101ng/ml
Second Trimester -17.2 - 270 ng/ml
Third Trimester 67.9 - 419 ng/ml

Clinical Significance :
- Prolactin is a hormone which is secreted in pulsatile manner and is also influenced by a variety of physiological stimuli like - stress, pain, coitus, nipple stimulation, sleep etc . Hence it is recommended to test 3 specimens at 20-30 minute intervals after pooling if clinically indicated.
- prolactin levels may show elevation if collected <3-4 hrs after waking up
- Prolactin test is used in diagnosis and management of pituitary adenomas, infertility, male and female hypogonadism etc
- Macroprolactin assay is recommended if prolactin levels are elevated but there are no signs and symptoms of hyperprolactinemia or if pituitary imaging studies are normal.
- Prolactin levels also show interference with certain psychiatric medicines, antihypertensives, opiates, ranitidine etc
- Results obtained after to interpreted in conjunction with clinical history and other findings

Method : Fully Automated Electrochemiluminescence Sandwich Immunoassay

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
CALCIUM	PHOTOMETRY	9.28	mg/dL	8.6 - 10.3
PHOSPHOROUS	**PHOTOMETRY**	**10.27**	**mg/dL**	**2.4 - 5.1**

Please correlate with clinical conditions.

Method :

CALC - Photometric test using Arezano III
PHOS - UNREDUCED PHOSPHOMOLYBDATE METHOD

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		Sample Type | Barcode	: SERUM | FN883087

TEST NAME	TECHNOLOGY	VALUE	UNITS
MAGNESIUM	**PHOTOMETRY**	**2.61**	**mg/dL**

Bio. Ref. Interval. :-

1.9 - 2.5

Clinical significance:

Magnesium is the fourth most abundant cation in the body and second most prevalent intracellular cation. The total body magnesium content is about 25 g or approximately 1 mol, of which 55% reside in the skeleton. About 45% of the magnesium is intracellular. In general higher the metabolic activity of cell, the greater is its magnesium content. Magnesium is a cofactor for more than 300 enzymes in the body.

Disorders of magnesium metabolism are separated into those causing hypomagnesaemia/magnesium deficiencies and hypermagnesemia. Hypomagnesaemia is common in patient in hospitals. Moderate to severe deficiency of magnesium is usually due to loss of magnesium from the gastrointestinal (gi) tract or kidneys. One of the more serious complications of magnesium deficiency is cardiac arrhythmia. Symptomatic hypermagnsemia is almost always caused by excessive intake, resulting from administration of antacids, enemas, and parenteral fluids containing magnesium. Depression of neuromuscular system is the most common manifestation of magnesium intoxication.

Please correlate with clinical conditions.
Method:- Photometric test using "" Xylidyl Blue""

Tests Done : JAANCH - WOMENS HAIRFALL SCREENING ADVANCED

Report Remarks : LabCode:1509040738/FE254

Dr.Vinitha Jose
MD(Path)

Scan QR to verify(valid for 30 days from release time)

Patient Name	: KANAGAVALLI K (21Y/F)	**Sample Collected on (SCT)**	: 15 Sep 2026 11:12
Referred By	: SELF	**Sample Received on (SRT)**	: 15 Sep 2026 11:53
Address	: INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007	**Report Released on (RRT)**	: 16 Sep 2026 09:05
		Sample Type | Barcode	: SERUM | FN883087

TEST NAME	TECHNOLOGY	VALUE	UNITS
C-REACTIVE PROTEIN (CRP)	IMMUNOTURBIDIMETRY	0.61	mg/L

Bio. Ref. Interval. :-

Acute phase determination : < 5 mg/L

Clinical Significance:
It's a protein present in the sera of acutely ill patients that bound cell wall C-polysaccharide of streptococcus pneumoniae and agglutinates the organisms. CRP is one of the strongest acute -phase reactants, with plasma concentrations rising up after myocardial infarction, stress, trauma, infection, inflammation, surgery, or neoplastic proliferation.

Concentrations >5 mg/L suggest the presence of an infection or inflammatory process. Concentrations are generally higher in bacterial than viral infection. The increase in peak is proportional to tissue damage. Determination of CRP is clinically useful to screen activity of inflammatory diseases such as rheumatoid arthritis; SLE;Leukemia;after surgery;to detect rejection in renal allograft recipients;to detect neonatal septicemia and meningitis. However, it is a nonspecific marker and cannot be interpreted without other clinical information.

Reference:
Tietz Textbook of clinical chemistry and molecular diagnosis fifth edition chapter 21 P538-539

Please correlate with clinical conditions.

Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

Tests Done : JAANCH - WOMENS HAIRFALL SCREENING ADVANCED

Report Remarks : LabCode:1509040738/FE254

Dr.Vinitha Jose
MD(Path)

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Patient Name	: KANAGAVALLI K (21Y/F)	**Sample Collected on (SCT)**	: 15 Sep 2026 11:12
Referred By	: SELF	**Sample Received on (SRT)**	: 15 Sep 2026 11:53
Address	: INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007	**Report Released on (RRT)**	: 16 Sep 2026 09:05
		Sample Type | Barcode	: SERUM | FN883087

TEST NAME	TECHNOLOGY	VALUE	UNITS
CORTISOL	C.L.I.A	13.78	µg/dL

Bio. Ref. Interval. :-

6.00am to 10am: 6.7 - 22.6 µg/dL
4.00pm to 8pm: <10.00 µg/dL

Clinical Significance:

- Cortisol is the major glucocorticoid product of the adrenal cortex.
- Cortisol plays an important role in the regulation of many physiological processes like energy metabolism, maintaining electrolyte balance and blood pressure, immunomodulation and stress responses, cell proliferation as well as cognitive function.
- Elevated serum levels of cortisol can also be seen in stress responses, psychiatric diseases, obesity, diabetes, alcoholism, contraceptives, estrogen therapy and pregnancy which may cause diagnostic problems in patients with Cushings Syndrome.
- Cortisol secretion is controlled by Adrenocorticotropic hormone (ACTH) of pituitary gland
- Normally, the Cortisol secretion happens in the second half of the night with peak cortisol production occurring in early morning, cortisol levels then decline throughout the day with lowest levels during first half of the night.
- Therefore circadian variations of cortisol release and influence of stress should be considered while sampling for cortisol test
- Samples should not be taken from, patients receiving biotin therapy, until at least 8 hours following the last biotin administration
- In some cases, interference is seen due to presence of antibodies to analyte-specific antibodies, streptavidin or ruthenium (in case of ECLIA method) can occur and interference with heterophile and HAMA antibodies can occur (in case of CLIA method assay)
- Cortisol is best measured in the morning when evaluating for possible Adrenal Insufficiency and best measured in the afternoon or evening to differentiate normal and Cushings Syndrome subjects.
- Increased Cortisol levels seen in pathologies like : Cushings Syndrome, Ectopic ACTH syndrome, prednisolone therapy, Ectopic CRH syndrome, Adrenal adenoma / carcinoma, Adrenal micronodular dysplasia, Adrenal macronodular hyperplasia, Stress
- Decreased Cortisol Levels seen in pathologies like - Addisons disease, Pituitary dysfunction
- The results should be interpreted in conjunction with clinical history and other findings

References: Kit insert

Please correlate with clinical conditions.

Method:- Competitive binding Immunoenzymatic assay

Tests Done : JAANCH - WOMENS HAIRFALL SCREENING ADVANCED

Report Remarks : LabCode:1509040738/FE254

Saakshi

Dr Saakshi Mittal
MD(Path)

Scan QR to verify(valid for 30 days from release time)

Patient Name	: KANAGAVALLI K (21Y/F)	**Sample Collected on (SCT)**	: 15 Sep 2026 11:12
Referred By	: SELF	**Sample Received on (SRT)**	: 15 Sep 2026 11:53
Address	: INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007	**Report Released on (RRT)**	: 16 Sep 2026 09:05
		Sample Type | Barcode	: SERUM | FN883087

TEST NAME	TECHNOLOGY	VALUE	UNITS
ANTI NUCLEAR ANTIBODIES (ANA)	E.L.I.S.A	0.05	OD Ratio

Bio. Ref. Interval. :

Negative: <0.90
Equivocal : 0.90 - 1.10
Positive: >1.11

Clinical Significance:
Anti-nuclear antibodies (ANA) are autoantibodies directed against nuclear components and are commonly associated with autoimmune diseases such as systemic lupus erythematosus (SLE), mixed connective tissue disease (MCTD), systemic sclerosis, Sjögren's syndrome, and rheumatoid arthritis.

However, ANA positivity is not specific to any one condition and may also be seen in healthy individuals, especially the elderly, or in cases of infections, malignancies, or drug-induced autoimmunity.

Equivocal results may require repeat testing or additional evaluation with specific antibody panels (e.g., anti-Sm, anti-RNP, anti-Ro, anti-La) for diagnostic clarification. A negative ANA test does not completely rule out autoimmune disease, particularly in early stages or when non-ANA-specific autoantibodies are involved. The final diagnosis should be made by the physician based on the overall clinical picture and appropriate correlation with additional investigations.

Note: Hemolysis, lipemia, icterus, microbial contamination, high-dose biotin, or heterophile antibodies may interfere with test accuracy and may lead to false-positive or false-negative results.

Method : INDIRECT SOLID PHASE IMMUNOASSAY

Please correlate with clinical conditions.

Tests Done : JAANCH - WOMENS HAIRFALL SCREENING ADVANCED

Report Remarks : LabCode:1509040738/FE254

Dr.Vinitha Jose
MD(Path)

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Patient Name	: KANAGAVALLI K (21Y/F)	**Sample Collected on (SCT)**	: 15 Sep 2026 11:12
Referred By	: SELF	**Sample Received on (SRT)**	: 15 Sep 2026 11:54
Address	: INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007	**Report Released on (RRT)**	: 15 Sep 2026 12:45
		Sample Type | Barcode	: EDTA Whole Blood | EZ153335

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	**SLS-Hemoglobin Method**	**9.7**	**g/dL**	**12.0-15.0**
Hematocrit (PCV)	CPH Detection	38.7	%	36.0-46.0
Total RBC	**HF & EI**	**4.87**	**X 10^6/µL**	**3.8-4.8**
Mean Corpuscular Volume (MCV)	**Calculated**	**79.5**	**fL**	**83.0-101.0**
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**19.9**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	**Calculated**	**25.1**	**g/dL**	**31.5-34.5**
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**60.5**	**fL**	**39.0-46.0**
Red Cell Distribution Width (RDW - CV)	**Calculated**	**21.4**	**%**	**11.6-14.0**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	349.3	-	*Refer Note below
MENTZER INDEX	Calculated	16.3	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	8.38	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	50.3	%	40-80
Lymphocytes Percentage	Flow Cytometry	35.4	%	20-40
Monocytes Percentage	Flow Cytometry	6.2	%	2-10
Eosinophils Percentage	**Flow Cytometry**	**6.7**	**%**	**1-6**
Basophils Percentage	Flow Cytometry	1.1	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	4.22	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.97	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.52	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.09	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	**Calculated**	**0.56**	**X 10^3 / µL**	**0.02 - 0.5**
Immature Granulocytes (IG)	Calculated	0.03	X 10^3 / µL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	376	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	9.1	fL	6.5-12
Platelet Distribution Width (PDW)	**Calculated**	**9.4**	**fL**	**9.6-15.2**
Platelet to Large Cell Ratio (PLCR)	**Calculated**	**17.9**	**%**	**19.7-42.4**
Plateletcrit (PCT)	Calculated	0.34	%	0.19-0.39

Remarks : Alert!!! RBCs:Marked anisocytosis moderate poikilocytosis. Predominantly normocytic normochromic with microcytes & ovalocytes.
Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

Tests Done : HEMOGRAM - 6 PART (DIFF)

Report Remarks : LabCode:1509074825/FE254

Dr.Vinitha Jose
MD(Path)

Scan QR to verify (valid for 30 days from release time)

CUSTOMER DETAILS

As declared in our data base

Name: KANAGAVALLI K **Age:** 21Y **Sex:** F **Customer Mobile No:** 9894377521

Barcodes/Sample_Type	:	FN883087(SERUM), EZ153335(EDTA)
Labcode	:	1509040738, 1509074825
Ref By	:	SELF
Sample_Type/Tests	:	SERUM:JAANCH - WOMENS HAIRFALL SCREENING ADVANCED EDTA:HEMOGRAM - 6 PART (DIFF)
Address	:	INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007
Sample Collected on (SCT)	:	15 Sep 2026 11:12
Report Released on (RRT)	:	16 Sep 2026 09:05
Amount Collected	:	**Rs.3530/-(three thousand five hundred and thirty only)**

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CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient, as the reported results are dependent on individual assay method(s) used/ equipments used and quality of specimen received.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
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EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95 % of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at customersupport@thyrocare.com or call us on 022-3090 0000.



*T&C Apply, *As on 5th December 2024
*As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited